

Syncope / Near Syncope / Weak and Dizzy

History

- Did patient lose consciousness completely or have near loss of consciousness?
- Precipitant factors – fatigue, sleep/food deprivation, heat, alcohol/drug ingestion, pain, fear?
- Activity prior to episode? (Sitting, standing up suddenly, exertion, urinating / defecating? Etc.)
- Did patient have preceding symptoms? (Chest pain? Shortness of breath? Nausea? Headache? Seizure aura? Back pain?)
- Did patient fall? Possible traumatic injuries?
- Possible seizure? Does patient appear post-ictal?
 - Patient with syncope may appear to have seizure-like activity but will not be post-ictal.
- Past medical history, medications, recent changes in medication or activity

Physical

- Vital signs – hypertension, hypotension, tachycardia, bradycardia, fever, hypoxia
- Blood glucose
- Mental status – Altered? Post-ictal?
- Maintaining airway? Maintaining oxygenation?
- EKG changes – Arrhythmia? STEMI? Pacemaker dysfunction?
- Traumatic injuries – Neck? Back? Head? Shoulder?
- Signs of CHF – respiratory difficulty, rales, edema?
- Neurologic deficit? Abnormality on Stroke Exam?

Serious Causes of Syncope / Near Syncope:

- Cardiac arrhythmias
- Myocardial Infarction
- Aortic dissection (AAA rupture)
- Pulmonary embolus
- Stroke / Intracerebral hemorrhage
- Medication overdose
- Toxic ingestion
- Infection / Sepsis
- Hypoglycemia
- Hypotension
- GI bleed
- Dehydration
- Anemia
- Ectopic pregnancy

Remember:

- All episodes of near-syncope (presyncope) should be treated as if the patient had syncope – the causes are the same!
 - Near-syncope / presyncope is defined as a state of lightheadedness, muscular weakness, blurred vision, and feeling faint
- All syncope / near-syncope should be considered a potentially life-threatening event and treated seriously.

EMT/AEMT

- Follow General Medical Care Guideline
- Follow Adult Resuscitation Guideline
- Follow appropriate Guideline of associated complaint

EMT/AEMT - Expanded Scope

- **NS/LR** 500ml IO Bolus for significant hypotension unable to tolerate fluid by mouth

Paramedic

- Advanced airway as indicated
 - 12 – Lead EKG
- Postural Hypotension (Positive Orthostatic Vital Signs)
- **NS/LR** 500 – 2000 mL IO/IV bolus to maintain SBP > 90